

Apollo Speciality Hospital

21, Greams Road, Chennai - 600006, Tamil Nadu
Ph: 044-28293333 | Email: records@apollochennai.org
Hospital Reg No: TN/CHE/2003/00412 | PMJAY Empanelment ID: H001

DISCHARGE SUMMARY

Summary No: APH/DS/2026/TCNEW02 Date: 29/06/2026

SECTION 1 — HOSPITAL & PATIENT INFORMATION

Hospital Name	Apollo Speciality Hospital
Address	21, Greams Road, Chennai - 600006, Tamil Nadu
PMJAY Empanelment ID	H001
Patient Name	Aditya Suresh
Age / Gender	12 Years / Male
Guardian	Mr. Suresh Krishnamurthy (Father)
PMJAY Beneficiary ID	P002
PMJAY Card No.	TN-PMJAY-2024-002567
Address	34, Anna Nagar East, Chennai - 600102

SECTION 2 — ADMISSION & DISCHARGE DETAILS

Date & Time of Admission	25/06/2026 at 10:00 hrs
Date & Time of Discharge	29/06/2026 at 12:00 hrs
Total Duration of Stay	4 Days (pre-auth indicative: 3 Days)
Reason for Extended Stay	Post-operative pain management — patient required additional IV analgesics and observation on Day 3 due to throat pain and low-grade fever. Extended by 1 day on clinical grounds.
Type of Admission	Planned / Elective
Ward / Room Category	General Ward
Treating Consultant	Dr. Meenakshi Sundaram
Qualification	MBBS, MS (ENT), DLO
Registration Number	TN-MED-44821
Specialty	ENT (Ear, Nose & Throat)
PMJAY Package Booked	SL016A — Tonsillectomy (U/L tonsillectomy unilateral/bilateral)
Pre-Auth Reference	PMJAY-PREAUTH-2026-TCNEW02-001

SECTION 3 — CLINICAL SUMMARY

Presenting Complaints:

Recurrent sore throat with high fever, 8-9 episodes per year for 3 years. Difficulty swallowing. Habitual snoring and mouth breathing at night.

Relevant Medical History:

Recurrent tonsillitis since age 9. No other chronic illnesses. No known drug allergies. No previous surgeries.

Pre-operative Examination:

BP 104/66 mmHg, Pulse 84/min, SpO2 99%, afebrile. Grade III bilateral tonsillar hypertrophy, cryptic and congested. X-ray neck lateral: adenoid-nasopharynx ratio 0.7 — adenoid hypertrophy confirmed.

SECTION 4 — DIAGNOSIS

Provisional Diagnosis (Admission)	Chronic Recurrent Tonsillitis, Grade III Bilateral Tonsillar Hypertrophy with Adenoid Hypertrophy — ICD-10: J35.0 / J35.3
Final Diagnosis (Discharge)	Post-operative status following bilateral tonsillectomy and adenoidectomy for chronic recurrent tonsillitis with Grade III tonsillar hypertrophy and adenoid hypertrophy — ICD-10: J35.3

SECTION 5 — PROCEDURES PERFORMED

DEVIATION NOTE: Pre-authorisation was obtained for bilateral tonsillectomy (SL016A). During surgery under GA, significant adenoid hypertrophy was confirmed on direct visualisation (consistent with pre-operative X-ray finding of adenoid-nasopharynx ratio 0.7). Adenoidectomy was performed in the same sitting as it was clinically necessary to address the obstructive adenoid tissue. This is an intraoperative clinical decision deviation from the pre-authorised procedure.

Procedure 1 (Pre-authed)	Bilateral Tonsillectomy — PMJAY Package SL016A
Procedure 2 (Additional)	Adenoidectomy — performed intraoperatively, not separately pre-authed
Date of Surgery	26/06/2026 at 09:30 hrs
Duration	75 minutes (combined)
Anaesthesia	General Anaesthesia — Dr. Kavitha Rajan, MD Anaesthesiology, Reg: TN-MED-31045
Technique — Tonsillectomy	Dissection and snare technique. Bilateral tonsils removed. Haemostasis achieved with bipolar cautery.
Technique — Adenoidectomy	Adenoid tissue curetted under direct vision. Haemostasis achieved. Specimens sent for histopathology.
Intraoperative Findings	Bilateral Grade III tonsils — severely hypertrophied, cryptic, adherent to pillars. Adenoids: large, obstructing posterior choanae bilaterally — adenoidectomy clinically necessary. No intraoperative complications.

SECTION 6 — POST-OPERATIVE COURSE

Day 1 (26/06/2026): Comfortable post-op, vitals stable, IV fluids and IV analgesics. Throat pain expected and managed with IV Paracetamol and Tramadol.

Day 2 (27/06/2026): Afebrile, tolerating oral liquids, throat pain persisting — IV analgesics continued. No bleeding from operative site.

Day 3 (28/06/2026): Low-grade fever (99.2F), throat pain — clinical decision to extend stay by 1 day for IV antibiotics and observation. No signs of secondary haemorrhage.

Day 4 (29/06/2026): Afebrile, throat pain improving, tolerating soft diet, declared medically fit for discharge.

SECTION 7 — COMPLICATIONS

No intraoperative complications. Post-operative: prolonged throat pain and low-grade fever on Day 3 necessitating 1-day extension of hospital stay. No secondary haemorrhage. No airway compromise.

SECTION 8 — CONDITION AT DISCHARGE

Stable. Afebrile. Throat pain reducing. Tolerating soft diet. Operative sites healing well. No active bleeding.

SECTION 9 — DISCHARGE MEDICATIONS

#	Medication	Dose	Frequency	Duration
1	Tab. Amoxicillin-Clavulanate 375mg	1 tablet	Twice daily	5 Days
2	Tab. Paracetamol 500mg	1 tablet	Three times daily	5 Days
3	Tab. Ibuprofen 200mg	1 tablet	Twice daily after food	3 Days
4	Syp. Pantoprazole 20mg	5 ml	Once daily before breakfast	7 Days
5	Throat Gargles (Betadine diluted)	As directed	Twice daily	7 Days

SECTION 10 — FOLLOW-UP INSTRUCTIONS

Day 7 (06/07/2026): Post-operative wound check at ENT OPD.

Day 14 (13/07/2026): Review — histopathology report collection.

Diet: Cold soft diet for 2 weeks. Avoid hot, spicy, hard foods for 3 weeks.

Activity: Rest at home for 1 week. Avoid strenuous activity for 3 weeks. No school for 10 days.

Warning Signs: Report immediately for any fresh bleeding from mouth/nose, high fever (>101F), or difficulty breathing.

SECTION 11 — DOCUMENTS SUBMITTED WITH THIS DISCHARGE SUMMARY

The following documents are available and submitted for PMJAY claim processing:

#	Document	Status
1	Discharge Summary (CAM Annexure 6 format)	Available — this document
2	Operative Notes (bilateral tonsillectomy + adenoidectomy)	Available
3	Pre-Anaesthesia Check-up Report (ASA Grade I, dated 24/06/2026)	Available
4	Post-operative Clinical Photograph (operative site Day 1)	Available
5	Intraoperative Clinical Photograph (tonsil and adenoid specimen)	Available
6	Gross Specimen Photograph (removed tonsils and adenoid tissue)	Available
7	Histopathology Report (tonsil and adenoid tissue — sent for HPE)	Available
8	Pre-operative Blood Reports (CBC, BT/CT, Blood Group, Sugar, ECG)	Available
9	X-Ray Neck Lateral View Report (adenoid hypertrophy confirmed)	Available
10	Indoor Case Papers / Clinical Notes	Available
11	Informed Consent Form (signed by parent/guardian)	Available
12	PMJAY Beneficiary Card Copy	Available
13	Patient Photo on Hospital Bed (admission)	Available
14	Anaesthesia Record	Available
15	Deviation Justification Letter — Adenoidectomy (intraoperative decision)	Available

SECTION 12 — TREATING DOCTOR'S DECLARATION

I certify that the patient was under my care, all treatments were medically necessary, procedures were performed as documented, and the extended stay was clinically warranted. The additional adenoidectomy was an intraoperative clinical decision based on direct visualisation confirming significant obstructive adenoid hypertrophy consistent with pre-operative imaging. All mandatory claim documents are enclosed.

Consultant ENT Surgeon

Dr. Meenakshi Sundaram
MBBS, MS (ENT), DLO
Reg: TN-MED-44821
Date: 29/06/2026
(Signed)

PMAM (PM-JAY Arogya Mitra)

Ms. Rani Selvam
PMAM ID: TN-PMAM-00934
(Signed)
Date: 29/06/2026

Guardian / Attendant

Mr. Suresh Krishnamurthy (Father)
(Signed)
Date: 29/06/2026